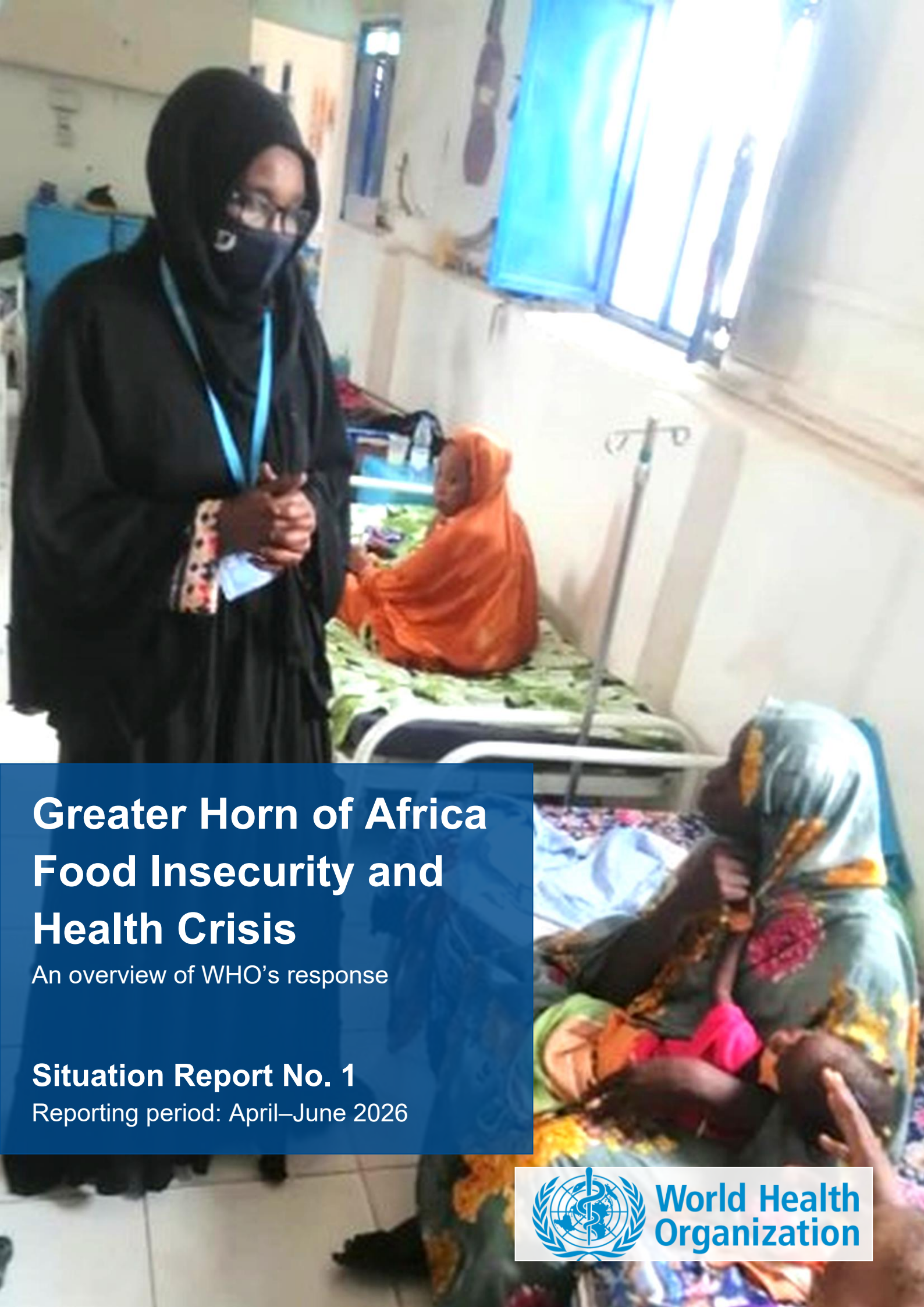




Greater Horn of Africa Food Insecurity and Health Crisis

An overview of WHO's response

Situation Report No. 1

Reporting period: April–June 2026



**World Health
Organization**

Greater Horn of Africa (GHOA) Food Insecurity and Health Crisis

An overview of WHO's response to the GHOA Food Insecurity and Health Crisis

Countries Covered: Djibouti, Ethiopia, Kenya, Somalia, South Sudan and Sudan

Situation Report No. 1 | Reporting period: April–June 2026 | Date of Publication: July 2026

Highlights

- ♣ The **Greater Horn of Africa (GHOA) Food Insecurity and Health Crisis** remains a **Grade 2 emergency** under the WHO Emergency Response Framework, affecting six countries across the WHO African Region and WHO Eastern Mediterranean Region. The crisis continues to be driven by the convergence of **acute food insecurity, conflict, climate shocks, population displacement, recurrent disease outbreaks, economic pressures and fragile health systems**.
- ♣ **More than 37.8 million people are experiencing or projected to experience acute food insecurity, while over 4.9 million children are projected to suffer acute malnutrition, including more than 1.5 million children requiring treatment for severe acute malnutrition (SAM)**. The overall regional burden is expected to be higher, as Ethiopia continues to report widespread food insecurity and increasing humanitarian needs, although a comparable national IPC Phase 3+ estimate is not yet available.
- ♣ **Acute food insecurity and malnutrition, compounded by recurrent disease outbreaks, conflict, displacement, climate shocks, economic pressures and reduced access to essential health services, continue to increase the risk of preventable illness and death**, particularly among children under five years of age, pregnant and lactating women, displaced populations, refugees, returnees, pastoralist communities and people living in hard-to-reach or insecure areas.
- ♣ WHO is providing **technical leadership** through a coordinated three-level response involving Headquarters, the WHO Regional Offices for Africa and the Eastern Mediterranean, and country offices, working closely with Ministries of Health and Health and Nutrition partners to strengthen emergency coordination, surveillance and early warning, outbreak preparedness and response, management of severe acute malnutrition with medical complications, continuity of essential health services, and integrated health sector coordination.
- ♣ During the reporting period, WHO supported the development of a regional Strategic Response Plan, strengthened surveillance and early warning systems, expanded support to Nutrition Stabilization Centres, reinforced outbreak preparedness and response, deployed technical expertise, pre-positioned emergency medicines and nutrition supplies, and supported integrated health and nutrition service delivery across priority countries.
- ♣ Significant operational challenges continue to constrain the response, including funding shortfalls, insecurity, restricted humanitarian access, overstretched health systems, shortages of trained health workers, supply chain disruptions and limited stabilization centre capacity, affecting the scale and sustainability of life-saving interventions.
- ♣ **WHO requires approximately US\$25.4 million over the next six months (July–December 2026)** to sustain priority life-saving health and nutrition interventions across the six-country response. Funding will support emergency coordination, surveillance and early warning, management of severe acute malnutrition with medical complications, continuity of essential health services, outbreak preparedness and response, and operational support to country-level implementation.

Regional Dashboard

Regional Situation at a Glance (April–June 2026)	
Countries affected	6
People experiencing or projected to experience acute food insecurity*	37.8 million+
Children affected or projected to suffer from acute malnutrition	4.9 million+
Children requiring treatment for severe acute malnutrition (SAM)	1.5 million+
WHO implementation period	July – December 2026
WHO Resource requirements	US\$25.4 million

*Estimate reflects currently available country data. The regional burden is expected to be higher, as Ethiopia continues to report widespread food insecurity without a directly comparable national IPC Phase 3+ estimate.

Situation Overview

The GHoA is experiencing one of the world's most complex humanitarian and public health emergencies. Across Djibouti, Ethiopia, Kenya, Somalia, South Sudan and Sudan, the convergence of acute food insecurity, conflict, climate-related shocks, displacement, recurrent disease outbreaks, economic pressures and fragile health systems continues to drive escalating humanitarian needs and threaten the health and survival of millions of people.

Although the underlying drivers differ across countries, the consequences are increasingly similar. Rising levels of acute food insecurity are contributing to worsening acute malnutrition, particularly among children under five years of age and pregnant and lactating women. At the same time, weakened health systems, disrupted essential health services, population displacement, poor water and sanitation conditions and constrained humanitarian access are increasing vulnerability to communicable diseases and preventable mortality.

Health systems across the region are under considerable pressure. Nutrition Stabilization Centres are reporting increasing admissions of children with severe acute malnutrition and medical complications, while many health facilities continue to face shortages of trained personnel, essential medicines, therapeutic commodities and operational resources. Insecurity, damaged infrastructure, flooding and logistical constraints continue to limit humanitarian access and timely delivery of life-saving services.

Countries are simultaneously responding to multiple epidemic-prone diseases, including cholera/acute watery diarrhoea, measles, malaria, dengue, mpox and other communicable diseases. Population movements, overcrowded displacement settings, interruptions to routine immunization, weak surveillance systems and limited access to safe water and sanitation continue to amplify public health risks.

WHO is providing technical leadership and operational support to Ministries of Health and Health and Nutrition partners through a coordinated regional response focused on emergency coordination, surveillance and early warning, outbreak preparedness and response, management of severe acute malnutrition with medical complications, continuity of essential health services and strengthening national response capacities. Particular emphasis is placed on protecting the most vulnerable populations while supporting countries to prevent excess morbidity and mortality and build resilience to evolving humanitarian and public health threats.

Regional Snapshot by Country

The table below summarizes the current humanitarian and public health situation across the six affected countries, highlighting the scale of need, acute malnutrition burden, principal public health risks and major operational constraints.

Country	Scale / affected population	Acute malnutrition	Key public health risks	Major operational constraints
Djibouti	230,000 people (22% of population) in IPC Phase 3+; Refugees, migrants and cross border populations remain highly vulnerable.	More than 34,000 children and pregnant/breastfeeding women affected by acute malnutrition; 7,700 children require urgent SAM treatment.	Cholera/AWD, polio alert, measles, malaria, dengue; cross-border disease risk.	Underfunded health system, non-functional/limited emergency systems, villages far from hospitals, WFP pipeline break, and shortage of essential supplies.
Ethiopia	Food insecurity remains widespread; 3.9 million IDPs and pastoral populations at very high risk; needs expected to peak during lean season.	High SAM burden and increasing inpatient cases; priority regions include Tigray, Amhara, Afar, Oromia, Somali, Sidama and southern/central areas.	Cholera/AWD, measles, malaria and other epidemic-prone diseases in areas with weak services.	Conflict and insecurity, access constraints, funding shortfalls, and critically constrained SAM-related supplies and medicines.
Kenya	An estimated 4.1 million people projected to face high levels of acute food insecurity (IPC Phase 3+), including 545,000 in Phase 4; an estimated 429,000 refugees are in IPC Phase 3+ including 186,000 refugees in emergency.	810,871 children (SAM: 202,230), aged 6-59 months in need for treatment for wasting and 116,800 pregnant/lactating women acutely malnourished; IPC AMN Phase 5 in Mandera, Turkana East/South and North Horr.	Measles, mpox, suspected anthrax, malaria, cholera, Dengue fever and flood-related water/vector-borne risks.	Simultaneous food insecurity and flooding, strained coordination, operational funding gaps for disease outbreak investigation and response, supply constraints of public health emergencies preparedness and insufficient financial resources to support cascade of 3 rd edition IDSR to sub-national level.
Somalia	6.6 million people in IPC Phase 3+; 30 drought-affected districts, with response prioritizing 13 districts; >500,000 people displaced due to drought.	1.88 million children projected to suffer acute malnutrition, including 493,000 likely SAM; Burhakaba at risk of famine and IPC AMN Phase 5.	Cholera/AWD, measles, diphtheria, polio, dengue, malaria and bloody diarrhoea, with increasing flood-related risks in riverine areas.	Limited health system functionality, funding cuts, access constraints, shipment delays, and HR gaps.
South Sudan	7.8 million people (55% of population) projected to face acute food insecurity Apr-Jul 2026; IPC Phase 5 pockets reported.	2.2 million children 6-59 months expected to face acute malnutrition; nearly 697,000 at risk of SAM; 1.2 million	Cholera, malaria, acute respiratory infections, and diarrhoeal disease risks,	Conflict, displacement, access constraints, non-functional/absent stabilization centres in priority counties and limited partner/resources.

Country	Scale / affected population	Acute malnutrition	Key public health risks	Major operational constraints
		pregnant/breastfeeding women need treatment. Ten counties classified in IPC in Upper Nile, Jonglei, and Unity.	compounded by conflict and flooding.	
Sudan	19.1 million people (41% of population analysed) projected to face acute food insecurity Feb-May 2026.	8.4 million people require nutrition assistance; >824,000 children under five with SAM and 122,700 with SAM and medical complications.	Cholera, measles, malaria and other communicable disease risk in the context of conflict, displacement and disrupted services.	Conflict and insecurity, damaged hospitals, high staff turnover, service quality gaps, reporting constraints, supply chain and funding gaps.

Priority Public Health Risks

- ♣ **Acute malnutrition:** Severe acute malnutrition and moderate acute malnutrition are increasing among children under five years of age and pregnant and lactating women. Several high-risk locations are reporting increased admissions, while stabilization capacity, referral pathways and supplies remain insufficient.
- ♣ **Outbreaks and epidemic-prone diseases:** Cholera/AWD, measles, malaria, mpox, dengue, diphtheria and other communicable diseases are occurring in settings affected by displacement, low immunity, poor WASH conditions, overcrowding and weak access to essential health services.
- ♣ **Disruption of essential health services:** PHC, immunization, maternal and child health services, IMCI, referral care, noncommunicable disease services and emergency care are affected by insecurity, facility closures, health workforce shortages, supply gaps and funding constraints.
- ♣ **Climate-related health risks:** Drought, flooding and rainfall variability are increasing water scarcity, food insecurity, displacement, vector-borne and waterborne disease risks, and pressure on already fragile health systems.
- ♣ **Protection and safeguarding risks:** Displaced, food-insecure and crisis-affected populations face heightened risks of gender-based violence, exploitation and abuse, limited access to services, and reduced ability to seek care early. Protection, safeguarding, GBV referral pathways, RCCE and PSEAH measures must remain integrated across the response.

WHO Strategic Priorities and Health Sector Response

WHO is working with partners to support national authorities through a coordinated three-level response involving headquarters, the WHO Regional Office for Africa, the WHO Regional Office for the Eastern Mediterranean and the six affected country offices.

Key areas of support include:

- ♣ **Emergency coordination:** WHO continues to coordinate the regional health response through country offices, regional level (AFRO and EMRO) and HQ, and, supporting Incident Management System structures, strategic coordination, technical guidance, regular three level coordination mechanisms, advocacy and resource mobilization.

- ♣ **Strategic and operational planning:** A Strategic Response Plan (SRP) has been developed to guide WHO's overall regional response to the GHoA Food Insecurity and Health Crisis. Country-level operational response plans are being used to define priority actions based on country-specific risks, needs, capacities and operational constraints.
- ♣ **Surveillance and early warning:** WHO is supporting countries to strengthen EWARS/IDSR, community-based surveillance, event-based surveillance, alert verification, outbreak investigation, reporting completeness, nutrition surveillance and use of mortality and health service data for operational decision-making.
- ♣ **Life-saving health and nutrition services:** WHO is supporting the management of severe acute malnutrition with medical complications through stabilization centres, mobile and outreach teams, capacity building of health workers, IMAM/IMCI training, referral pathways, supportive supervision and continuity of essential primary health care services.
- ♣ **Outbreak prevention and response:** WHO is supporting rapid response teams, outbreak investigation and response, preparedness and response to cholera/AWD, measles response, and mpox; malaria prevention and treatment, IPC/WASH measures, vaccination activities, laboratory support and sample referral systems.
- ♣ **Supplies and logistics:** WHO is supporting procurement, pre-positioning and delivery of emergency health kits, cholera kits, SAM-related medical supplies, diagnostics, essential medicines, laboratory supplies and other critical commodities, subject to available funding and operational access.
- ♣ **Community protection, RCCE and PSEAH:** WHO is supporting community engagement, RCCE, social listening, safeguarding measures, GBV referral pathways and integration of protection and PSEAH considerations into health and partner-delivered activities.
- ♣ **Resource mobilization and advocacy:** WHO is using the Strategic Response Plan, country operational plans, donor briefs and advocacy materials to mobilize resources and sustain essential life-saving interventions across the six-country response.

WHO Response at a Glance

During the reporting period, WHO supported national authorities and partners to:

- ♣ develop a regional Strategic Response Plan and support country-level operational planning;
- ♣ strengthen EWARS/IDSR, event-based surveillance, nutrition surveillance and outbreak alert verification;
- ♣ support stabilization centres for children with severe acute malnutrition and medical complications;
- ♣ train frontline health and nutrition workers on SAM case management, IMAM/IMCI, IDSR and related priority areas;
- ♣ support deployment of mobile and outreach teams to expand access to essential health and nutrition services;
- ♣ pre-position and deliver emergency health kits, cholera kits, SAM-related medical supplies and essential medicines;
- ♣ support outbreak preparedness and response for cholera/AWD, measles, malaria, mpox, dengue and other epidemic-prone diseases;
- ♣ strengthen RCCE, community engagement, safeguarding, GBV referral pathways and PSEAH integration;
- ♣ support resource mobilization and advocacy for urgent life-saving health and nutrition interventions.

Multi-Country Situation Map

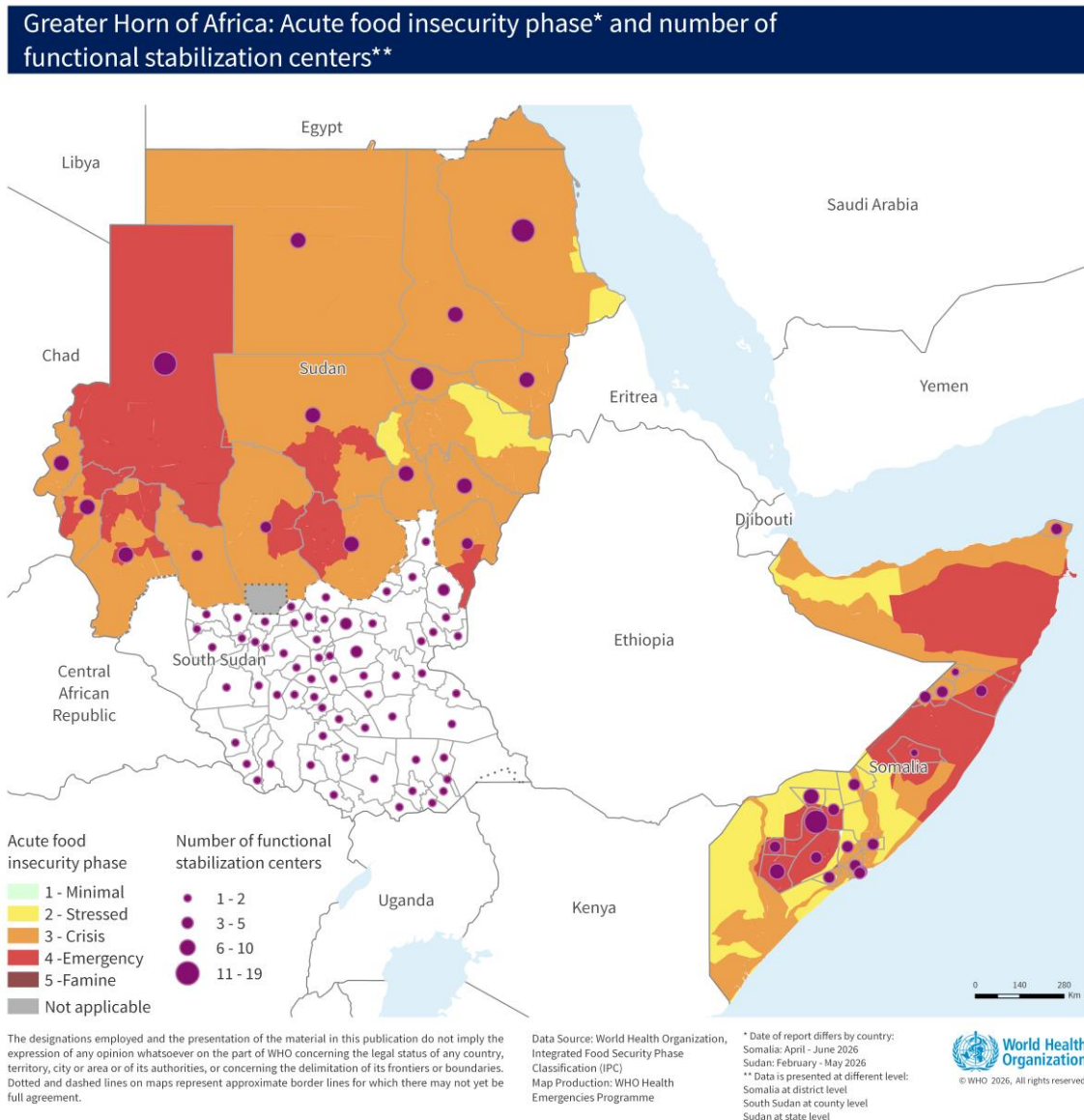


Figure 1. Greater Horn of Africa: Acute food insecurity phase and number of functional stabilization centres.

Figure 1 above shows the overlap between acute food insecurity and the availability of functional stabilization centres across the GHoA countries. The map highlights a major operational concern: several areas facing high levels of acute food insecurity and malnutrition have limited access to functional stabilization capacity for the management of severe acute malnutrition with medical complications.

This reinforces the need to prioritize support to stabilization centres, referral pathways, supply pre-positioning and health worker capacity in high-risk areas, particularly where food insecurity, conflict, displacement and disease outbreaks intersect.

Please refer to **Annex 1** for the individual country situation maps.

Country Operational Updates

Djibouti

Situation: Djibouti continues to experience the combined effects of prolonged drought, water scarcity, high food import dependence and increasing cross-border population movements. Approximately 230,000 people (around 22% of the population) are experiencing acute food insecurity (IPC Phase 3+), while more than 34,000 children and pregnant or breastfeeding women are affected by acute malnutrition. The country also remains vulnerable to cholera, malaria, measles, dengue and other epidemic-prone diseases due to population mobility, limited access to safe water and overstretched health services.

WHO Response: WHO, working closely with the Ministry of Health and Health and Nutrition partners, is strengthening emergency coordination, disease surveillance and outbreak preparedness while supporting the continuity of essential health and nutrition services. Current support includes strengthening EWARS/IDSR and cross border surveillance, improving nutrition surveillance, supporting stabilization centres for children with severe acute malnutrition (SAM) with medical complications.

Priorities: Expand surveillance coverage, strengthen stabilization center capacity and pre-positioning essential medicines and nutrition supplies, improve referral pathways and secure additional resources to sustain essential health and nutrition services.

Ethiopia

Situation: Food insecurity and acute malnutrition remain widespread across conflict- and drought-affected regions, with needs expected to increase during the lean season. Recurrent outbreaks of cholera, measles and malaria continue to place additional pressure on already strained health services.

WHO Response: WHO, in collaboration with the Ministry of Health and Health and Nutrition partners, is supporting mobile health and nutrition teams, strengthening stabilization centres, enhancing nutrition surveillance and providing technical support for the management of severe acute malnutrition. WHO also supports outbreak preparedness and response, Health and Nutrition Cluster coordination, frontline health worker training and distribution of PED-SAM kits and other essential medical supplies where operationally feasible.

Priorities: Scale up stabilization services, strengthen nutrition surveillance, improve access to conflict affected populations and ensure adequate supplies of therapeutic commodities and essential medicines.

Kenya

Situation: Kenya is facing the combined impacts of acute food insecurity, flooding and recurrent disease outbreaks, particularly in the arid and semi-arid lands (ASALs). Critical levels of acute malnutrition persist in several hotspot counties, while flooding continues to increase the risk of cholera, dengue, malaria and other epidemic-prone diseases.

WHO Response: WHO, together with the Ministry of Health and Health and Nutrition partners, is strengthening disease surveillance, emergency coordination and outbreak preparedness while expanding integrated health and nutrition outreach services in priority counties. Support includes rapid response teams, nutrition surveillance, community screening and referral, IPC/WASH interventions, risk communication and community engagement, and pre-positioning of emergency health supplies.

Priorities: Strengthen surveillance and preparedness, expand integrated outreach services, reinforce referral systems and sustain emergency health and nutrition supply pipelines.

Somalia

Situation: Somalia continues to experience severe drought-related humanitarian and public health impacts, with acute food insecurity and malnutrition affecting millions of people across priority districts. Recurrent outbreaks of cholera, measles, malaria and other communicable diseases continue to strain an already fragile health system and increase the demand for life-saving health and nutrition services.

WHO Response: WHO, working with the Ministry of Health and Health and Nutrition partners, continues to strengthen the management of severe acute malnutrition through Nutrition Stabilization Centres, expansion of Integrated Disease Surveillance and Response (IDSR), deployment of mobile health teams, active case finding and referral, capacity building of frontline health workers, and pre-positioning of emergency medicines, diagnostics and cholera supplies. WHO also supports integrated coordination to strengthen service delivery and outbreak response in priority districts.

Priorities: Expand mobile health services, strengthen stabilization centre capacity, sustain surveillance and ensure uninterrupted supplies and operational support for priority districts.

South Sudan

Situation: South Sudan continues to experience worsening food insecurity and acute malnutrition driven by conflict, displacement and flooding. Several priority counties continue to report high levels of severe acute malnutrition, while gaps remain in stabilization centre coverage, referral pathways and access to essential health services.

WHO Response: WHO, together with the Ministry of Health and Health and Nutrition partners, is strengthening stabilization centre functionality, improving referral pathways, building the capacity of frontline health workers and enhancing the quality of care for children with severe acute malnutrition and medical complications. WHO also supports nutrition surveillance, technical supervision and coordinated scale-up of nutrition services in priority counties.

Priorities: Restore non-functional stabilization centres, expand nutrition services in underserved counties, strengthen referral systems and ensure adequate therapeutic supplies and operational resources.

Sudan

Situation: Sudan remains the most severe food insecurity and nutrition emergency in the Greater Horn of Africa. Conflict, displacement and damaged health infrastructure continue to drive exceptionally high levels of acute malnutrition while severely disrupting access to essential health services and increasing the risk of excess morbidity and mortality.

WHO Response: WHO, in collaboration with the Federal Ministry of Health and Health and Nutrition partners, continues to provide technical leadership for the management of severe acute malnutrition through support to Nutrition Stabilization Centres, nutrition surveillance, frontline health worker training, supportive supervision, quality improvement, operational support to priority centres and pre-positioning of therapeutic supplies. WHO also supports coordinated planning and technical assistance to strengthen the quality and continuity of life-saving nutrition and health services despite significant operational constraints.

Priorities: Maintain stabilization centre operations, sustain therapeutic supply chains, strengthen surveillance and reporting, expand support to conflict-affected areas and mobilize additional funding for life-saving interventions.

Key Gaps and Challenges

- ♣ Insufficient and unpredictable funding is limiting the scale, speed and continuity of life-saving interventions across the six-country response.
- ♣ Health systems and WHO country teams are stretched by multiple concurrent emergencies, including outbreaks, flooding, conflict-related displacement and preparedness for emerging threats.

- ♣ Supply chains are constrained by funding shortages, pipeline breaks, shipping delays, increased fuel/transport costs, and limited stock of SAM-related kits and essential medicines.
- ♣ Stabilization centre functionality and referral pathways remain inadequate in several high-risk areas, especially where insecurity, displacement or facility damage restrict access.
- ♣ Surveillance and reporting systems require strengthening, including nutrition surveillance, mortality monitoring, outbreak alerts, health facility functionality and partner 4W data.
- ♣ Access constraints, insecurity and population movement continue to limit service delivery and increase the risk of under-detection of malnutrition and outbreaks.
- ♣ High staff turnover remains a challenge, requiring continuous capacity strengthening efforts.

WHO Resource Requirements (July–December 2026)

WHO requires **US\$25.4 million over the next six months (July–December 2026)** to sustain priority life-saving health and nutrition interventions across the Greater Horn of Africa. The requested resources will support implementation of the regional Strategic Response Plan and enable WHO to provide technical leadership, strengthen surveillance and outbreak preparedness and response, sustain essential health and nutrition services, support the management of severe acute malnutrition with medical complications, maintain emergency coordination, and reinforce operational support to country responses.

The budget also includes provision for regional and headquarters technical backstopping, surge deployments, information management, monitoring and evaluation, risk communication and community engagement (RCCE), prevention of sexual exploitation, abuse and harassment (PSEAH), and other cross-cutting functions required to support coordinated implementation across the six-country response.

Resource requirements will be refined as country operational plans evolve and additional needs are identified.

WHO Strategic Response Priorities	Estimated Budget (US\$ millions)
Emergency Coordination, Technical Leadership and IMST support (countries, AFRO Region, EMRO Region and HQ)	3.5
Surveillance, EWARS, Outbreak preparedness and response	5.0
Essential Health and Nutrition services including Nutrition Stabilization Centres and capacity building	9.7
Emergency supplies, logistics and operational support	4.7
Cross-cutting functions (Information Management, RCCE, PSEAH, Monitoring and Evaluation, surge deployments and technical backstopping)	2.5
Total (July – December 2026)	25.4

Outlook

Without sustained funding and operational support, the convergence of food insecurity, acute malnutrition, conflict, climate-related shocks, population displacement and recurrent disease outbreaks is expected to further increase humanitarian needs across the Greater Horn of Africa during the coming months. WHO will continue working with Ministries of Health and Health and Nutrition partners to strengthen surveillance, outbreak preparedness and response, management of severe acute malnutrition, continuity of essential health services and coordinated emergency operations to reduce preventable illness and death among the most vulnerable populations.

Annex 1 – Country Situation Maps

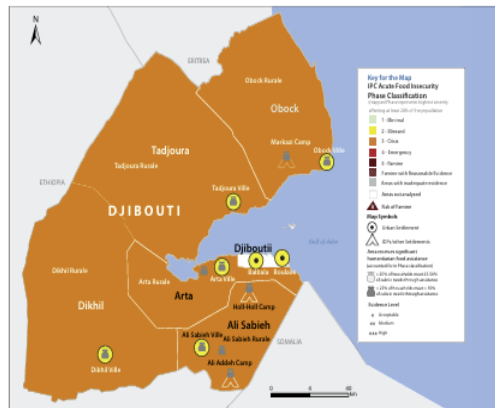


DJIBOUTI | ANALYSE DE L'INSÉCURITÉ ALIMENTAIRE AIGUË ET DE LA MALNUTRITION AIGUË DE L'IPC

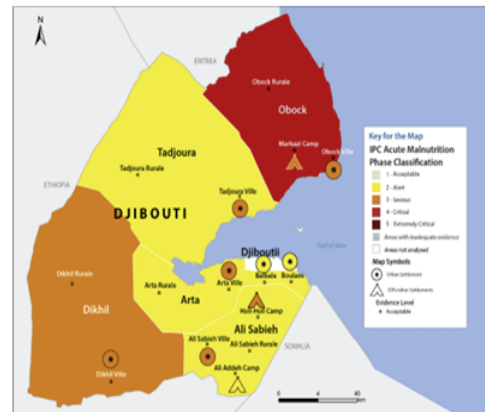
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ANNEXE 1: ANALYSE COMPARATIVE ENTRE LA SITUATION NUTRITIONNELLE ET L'INSECURITE ALIMENTAIRE AIGUE

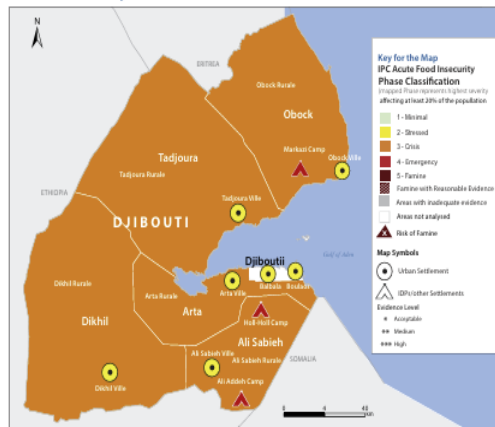
Situation Actuelle IPC AFI: Mai – Juin 2025



Situation Actuelle IPC MNA: Janvier – Juillet 2025



Situation Projetée IPC AFI: Juillet - Décembre 2025



Situation Projetée IPC AMN: Août - Décembre 2025

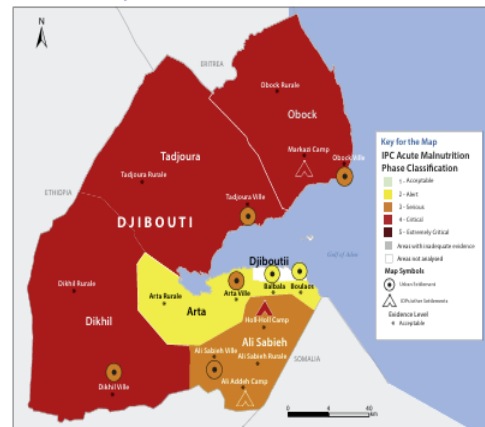
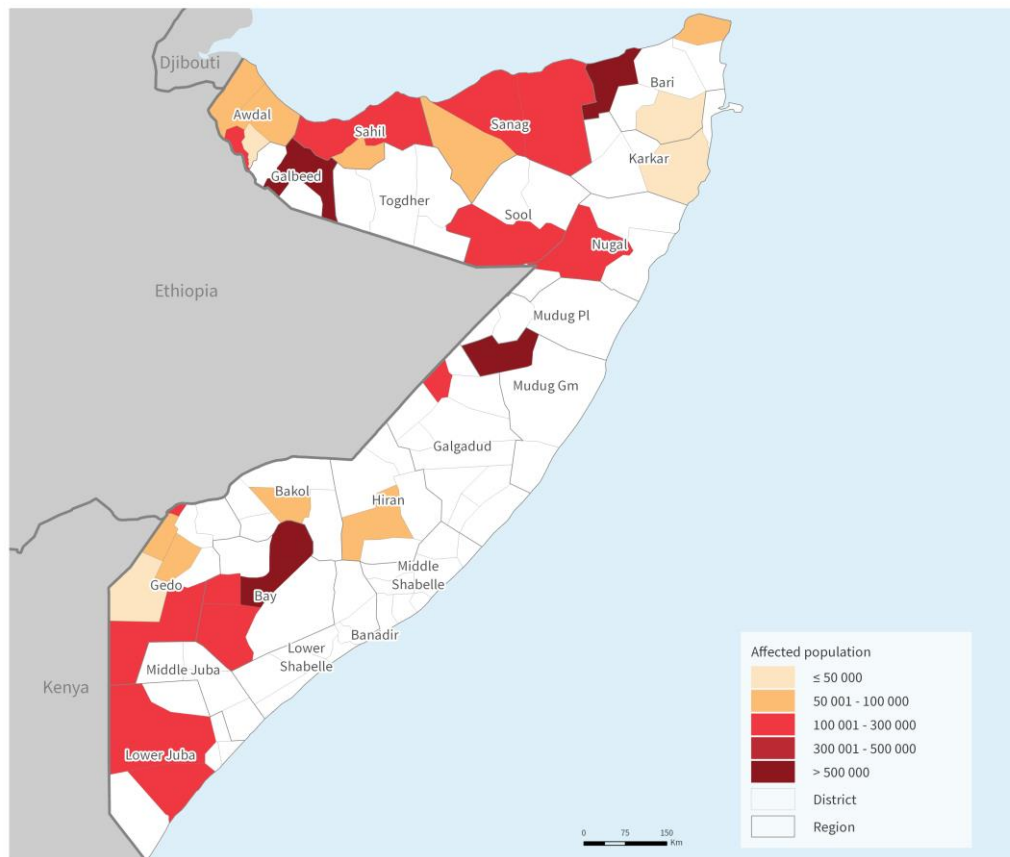


Figure A1. Djibouti IPC acute food insecurity and acute malnutrition classifications (January–December 2025).

Somalia: Affected population in drought - impacted districts

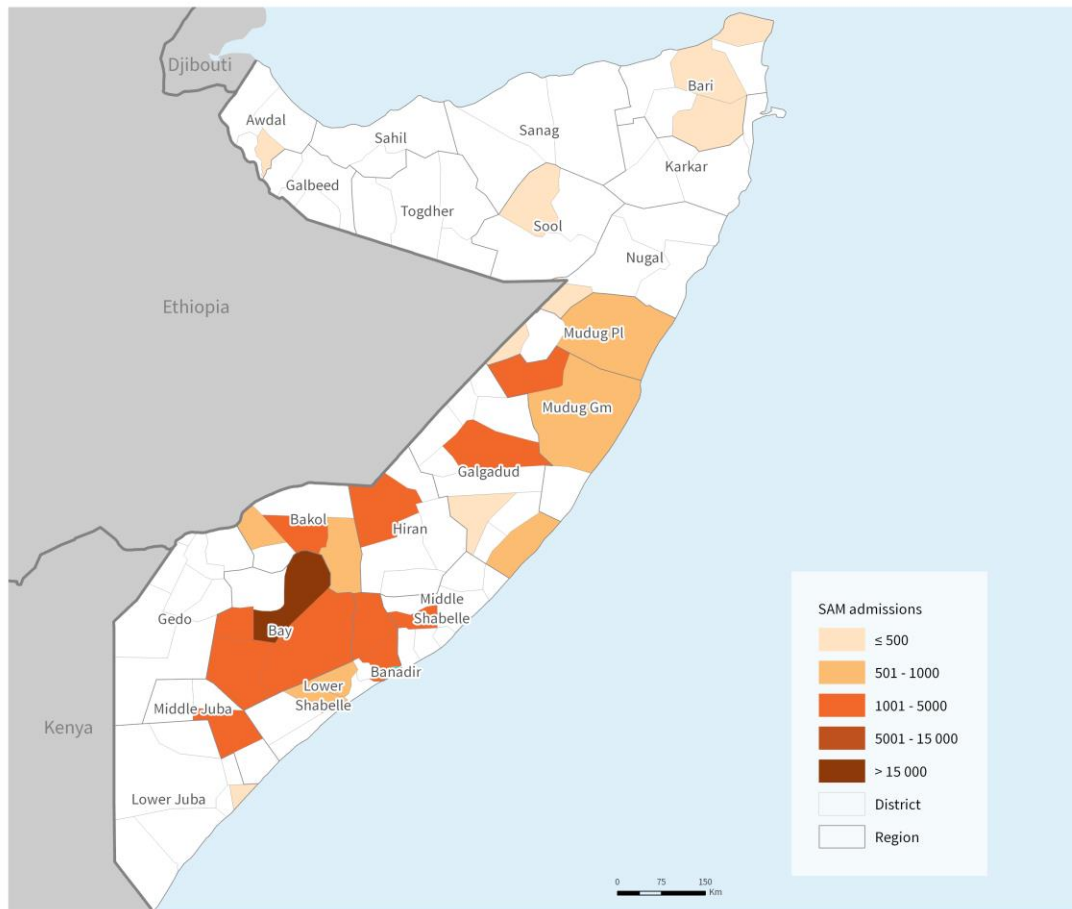


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Data Source: World Health Organization
Map Production: WHO Health Emergencies Programme
Map Date: 19 June 2026

Figure A2. Somalia affected population in drought-impacted districts map (2026).

Somalia: Severe Acute Malnutrition (SAM) admissions by district

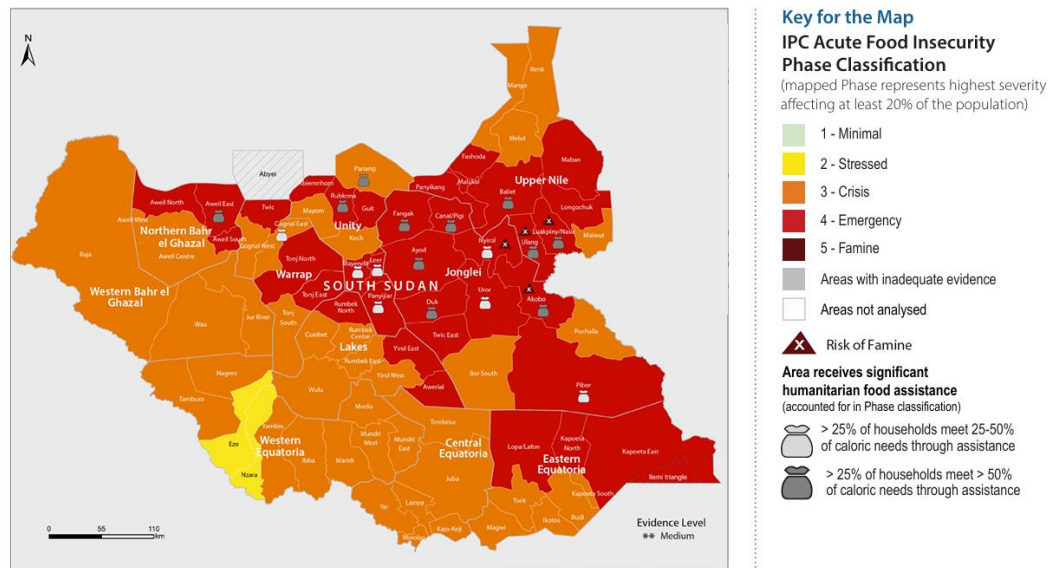


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Data Source: World Health Organization
Map Production: WHO Health Emergencies Programme
Map Date: 19 June 2026

Figure A3. Somalia severe acute malnutrition (SAM) admissions by district map (2026).

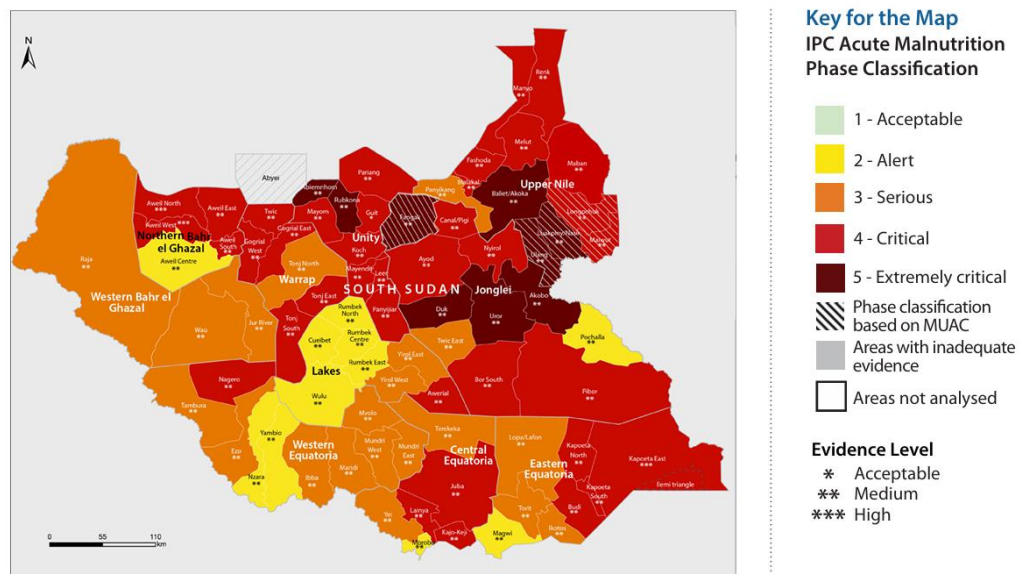
ACUTE FOOD INSECURITY PROJECTION UPDATE MAP AND POPULATION TABLE (APRIL - JULY 2026)



Disclaimer: the administrative boundaries and names shown and the designations used on this map do not imply official endorsement or acceptance by the United Nations. Final boundary between the Republic of Sudan and the Republic of South Sudan has not yet been determined. Final status of Abyei area is not yet determined.

Figure A4. South Sudan acute food insecurity projection update map and population table (April–July 2026).

ACUTE MALNUTRITION PROJECTION UPDATE MAP AND OVERVIEW (APRIL - JUNE 2026)



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Figure A5. South Sudan acute malnutrition projection update map and overview (April–June 2026).

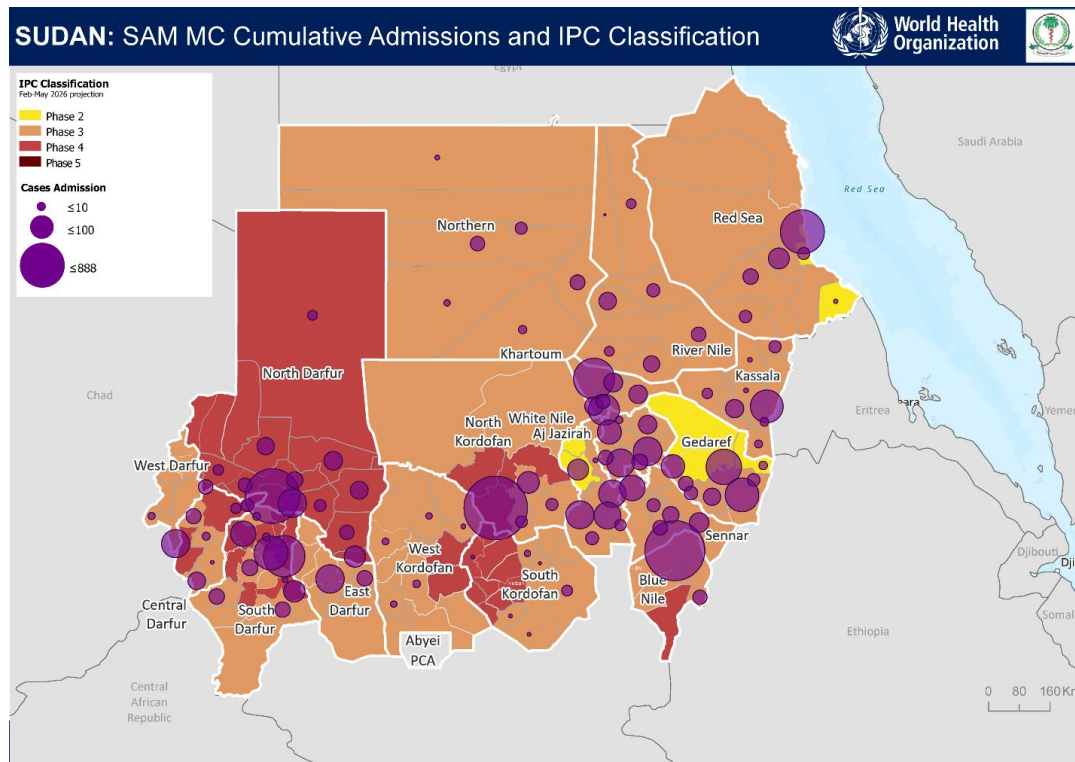


Figure A6. Sudan SAM with medical complications cumulative admissions and IPC classification (April 2026).

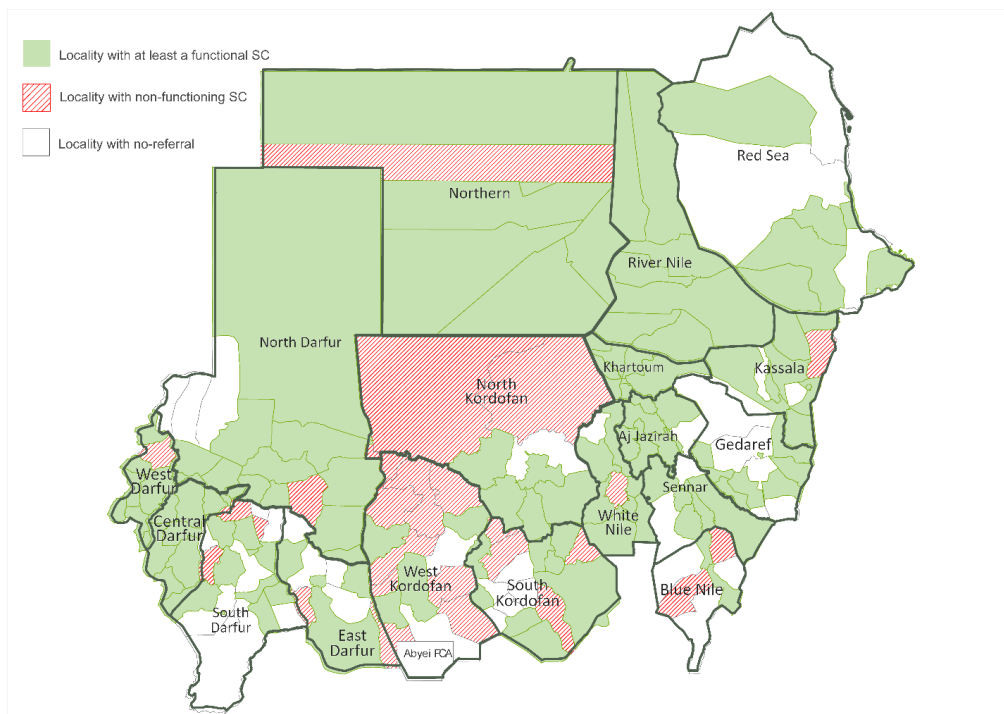


Figure A7. Sudan stabilization centre functionality (April 2026).